Second Victim Support System and Wellness Warriors: An Integrative Strategy for Safeguarding Healthcare Worker Well-being in Hospitals

Astri Mulyantini Monik[1], Vidya Cecilia[2]

[1] Dr. Cipto Mangunkusumo National General Hospital, Jakarta, Indonesia

[2] Occupational Medicine Residency Program, Faculty of Medicine, Universitas Indonesia, Jakarta, Indonesia

280
Healthcare workers identified and offered support

9/9
Sentinel event RCAs with second victim follow-up

136
Wellness Warriors trained across 57 units

8/11
International consensus indicators achieved

Background

- Patient safety incidents can turn involved healthcare workers (HCWs) into **second victims**: psychological distress, burnout, and loss of professional confidence.
- Most support programmes depend on **self-referral**, so the affected population remains unknown.
- International consensus now uses *support provided / second victims identified* as a core indicator,[1] yet reports from Southeast Asia are scarce.

Objective & Methods

To describe how Wellness Warriors are integrated with second victim management and root cause analysis (RCA), and to map programme performance against 11 international consensus indicators.

Design	Descriptive implementation study, document review
Setting	RSCM, Indonesia's national referral hospital (~5,300 staff, 57 units)
Period	January to August 2026
Framework	Scott's three-tiered model;[2] 25-item Indonesian SVEST;[3] 11 consensus indicators[1]

Wellness Warriors Support Pathway

TIER 1 · UNIT-LEVEL IDENTIFICATION — Quality Team & OSH Unit

1. **Sentinel / adverse event** occurs and an incident report is filed.
2. The **Quality Team** (Risk Management, Quality and Cost Control) conducts a **non-punitive RCA**.
3. The **OSH Unit** joins the RCA and identifies staff involved who may be second victims.
4. The OSH Unit sends **SVEST** to the unit and to the Wellness Warriors.
   OSH Unit ⟷ Wellness Warriors

TIER 2 · PEER SUPPORT — Wellness Warriors

5. Wellness Warriors give **Psychological First Aid (PFA)** and distribute SVEST to affected staff.
6. Staff complete SVEST and return it via Wellness Warriors to the OSH Unit.

TIER 3 · PROFESSIONAL SUPPORT — Occupational Medicine

7. **Occupational Medicine** reviews SVEST scores across 7 domains.

8a · All domains below cut-off
Report and close follow-up.

8b · Any domain ≥ cut-off
Occupational Medicine interview, then referral to a psychiatrist if needed.

9. **Reporting:** OSH Unit → Quality Team → Board of Directors.

OSH Unit: Environmental Health and Hospital Occupational Safety and Health Installation (IKL-K3RS) · PFA: Psychological First Aid · SVEST: Second Victim Experience and Support Tool

SVEST: 25 items, 7 domains

Likert 0 to 4. Positively worded support items are reverse-scored, so a higher score always means a worse condition. Cut-off = 75% of each domain maximum.

Psychological distress	≥12/16
Physical distress	≥12/16
Colleague support	≥12/16
Supervisor support	≥12/16
Institutional support	≥9/12
Non-work-related support	≥6/8
Professional self-efficacy	≥12/16

Results

280
identified and offered support

202
staff in 6 of 9 completed cases

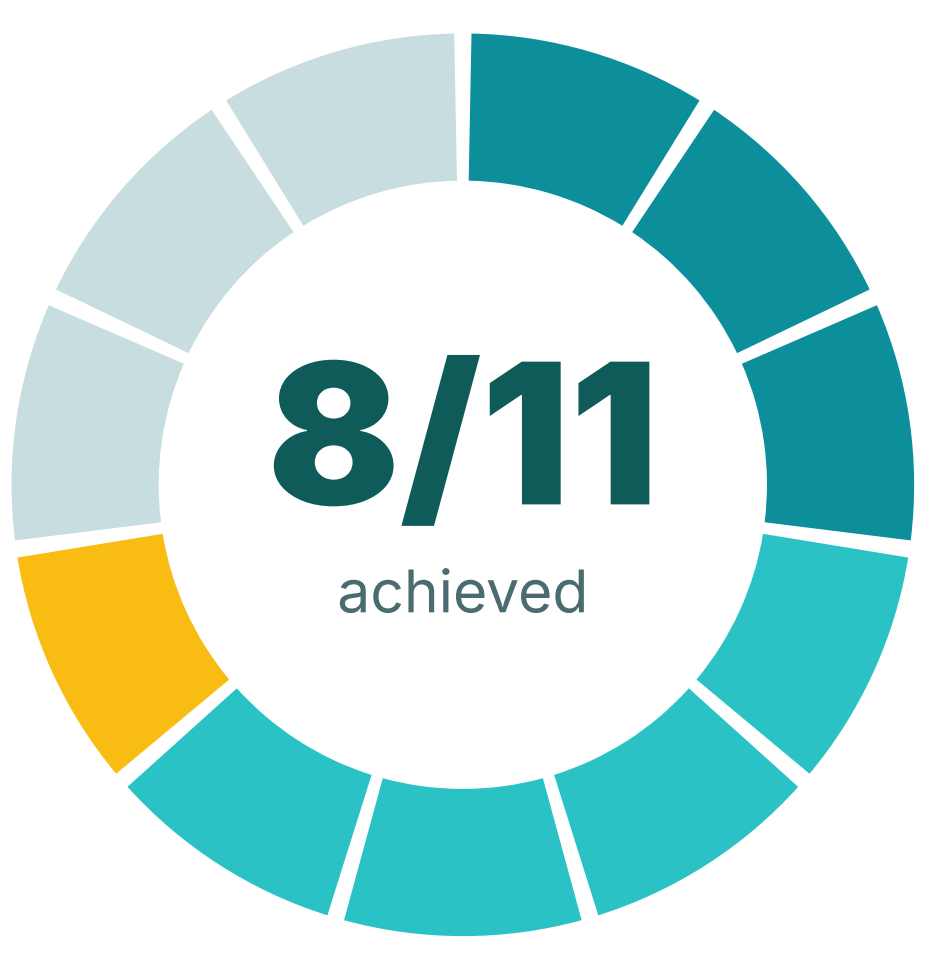
32
SVEST returned (15.8%)*

0
at or above any cut-off

- Median 32 HCWs per event (range 18 to 52)
- Most affected: **trainees and nurses**
- 3 of 9 cases still in progress
- Mean time from incident to first contact: **[___ days]**

*Response rate = 32 / 202 × 100% = 15.8%; data collection ongoing.

Consensus Indicators[1]



4 newly introduced (NEW)
- ■ Awareness
- ■ Process / structure
- ■ Impact
- ■ Not yet measured

AWARENESS

✓ Support / activation requests	9/9
✓ Support / SV identified NEW	280/280
✓ HCWs aware of programme	100%

PROCESS / STRUCTURE

✓ Time from incident to first contact NEW	[__ d]
✓ Approved institutional policy	SOP
✓ Trained / total peer supporters	136/136
✓ HCWs trained on SV topic NEW	57 units

IMPACT

- ✓ Perceived benefit after peer support: support from supervisors, colleagues, organisation
- ✓ Distress before and after support
- ✓ Qualitative feedback from SV
- ✓ Lost workdays, SV vs total NEW

Discussion

From self-referral to proactive case-finding. Linking second victim identification to every sentinel event RCA produced a known denominator of 280 HCWs across 9 events. This puts into practice the observation that institutions can screen at-risk professionals immediately after an event,[4] and makes the consensus indicator *support provided / second victims identified* measurable.[1]

Peer support first. In the SVEST validation study, the most desired form of support was a respected peer to discuss what happened.[3] Wellness Warriors (136 in 57 units) put this into practice as Tier 2, in line with the European recommendation for peer support programmes.[5]

Patient safety and worker well-being in one pathway. Embedding support in a non-punitive RCA connects incident learning with staff care and supports a just culture.[5]

Early SVEST data. Returns are still low (15.8%) because collection is ongoing, and no respondent has reached a domain cut-off so far. These early results should be interpreted with caution.

Next step: impact indicators. Second victim distress has been linked to turnover intention and absenteeism, mediated by organisational support.[6] Measuring distress before and after support, qualitative feedback, and lost workdays is the next priority.

Limitations. Single-centre document review.

Conclusion Integrating Wellness Warriors, second victim management, and system-based RCA establishes a non-punitive organisational framework. It can be implemented in hospitals with existing incident review structures and is a foundation for sustainable healthcare worker well-being.

References

1. Guerra-Paiva S, Carrillo I, Mira J, Fernandes J, Strametz R, Gil-Hernández E, et al. Developing core indicators for evaluating second victim programs: an international consensus approach. Int J Public Health. 2024;69:1607428.
2. Scott SD, Hirschinger LE, Cox KR, McCoig M, Hahn-Cover K, Epperly KM, et al. Caring for our own: deploying a systemwide second victim rapid response team. Jt Comm J Qual Patient Saf. 2010;36(5):233-40.
3. Burlison JD, Scott SD, Browne EK, Thompson SG, Hoffman JM. The Second Victim Experience and Support Tool: validation of an organizational resource for assessing second victim effects and the quality of support resources. J Patient Saf. 2017;13(2):93-102.
4. Scott SD, Hirschinger LE, Cox KR, McCoig M, Brandt J, Hall LW. The natural history of recovery for the healthcare provider "second victim" after adverse patient events. Qual Saf Health Care. 2009;18(5):325-30.
5. Mira J, Carillo I, Tella S, Vanhaecht K, Panella M, Seys D, et al. The European Researchers' Network Working on Second Victim (ERNST) policy statement on the second victim phenomenon for increasing patient safety. Public Health Rev. 2024;45:1607175.
6. Burlison JD, Quillivan RR, Scott SD, Johnson S, Hoffman JM. The effects of the second victim phenomenon on work-related outcomes: connecting self-reported caregiver distress to turnover intentions and absenteeism. J Patient Saf. 2021;17(3):195-9.